

Care Everywhere Document Pull Anomaly Analysis

Statistical Review of Organization-Level Connection Activity

(Daily-Grain, All-Organization Screen)

Ambulatory Data Integrity & Quality Analysis | The Health System | July 22, 2026

Staff 01, Data Scientist 1

Executive Summary

This report screens every organization connected to The Health System's Epic system via Care Everywhere for anomalous document-pull activity, using 179 million individual completed document-pull events at daily grain, bounded to July 1, 2023 through June 30, 2026.

Company 1 remains the most extreme statistical outlier in the network: a peak single-day z-score of 381.9 against a conventional outlier threshold of 3, a 31.6-times increase in average daily pull volume between an early baseline period and the most recent six months, and a sustained, unreverted step-change beginning in April 2025.

- Tier 1 (active, ongoing surges): Company 1, Company 2, and Company 3 (including two related connections, Company 4 and Company 5) show sustained, unreverted volume increases with no return to baseline.
- Tier 2 (resolved episodic anomalies): Company 6 and Company 7 (including the related Company 8 connection) show extreme, time-boxed spikes that have already ended.
- Tier 3 (connection lifecycle events): two organizations (Company 9, Company 10) show brief historical bursts consistent with onboarding backfills; three additional organizations newly identified in this revision (Company 11, Company 12, Company 13) show the inverse pattern - real historical volume that appears to have recently stopped.
- Tier 4 (newly identified, erratic high-volatility organizations): Company 14 and Company 15 show irregular, bursty monthly volume with no clear trend, a pattern distinct from both sustained surge and simple decline.
- Tier 5 (large-scale, non-anomalous organizations): Company 16 is a major new finding in this revision - a very high mean daily z-score (24.2) driven entirely by its scale relative to smaller network participants, not by anomalous growth. Company 17, Company 18, and Company 19 remain in this category from the original analysis.

A methodology correction is also reported here: the prior version's use of "share of pulls with no attached encounter type" as a standalone indicator of non-clinical or automated activity does not hold up once tested against the full organization list. Several of the largest participants in this system - including Company 20 and Company 21 - show unspecified-encounter rates of 97 to 100 percent, higher than Company 1's 66.1 percent. This metric is retained in this report only as a time-series check against known event timing, not as an absolute comparison across organizations.

1. Purpose and Scope

This analysis was initiated following an observed surge in document pulls attributed to Company 1 and was subsequently expanded to (a) recompute the original findings on a richer, row-level data source and (b) layer encounter type, document type, and patient demographic detail onto the volume findings so that the clinical or administrative justification for pull activity could be assessed alongside the raw statistics.

Scope note: this analysis covers organization type 1 ("Organization") connections only. Two other connection categories - Cosmos (Epic's research/benchmarking data pipeline, 277 million rows) and API Consumer (21.9 million rows) - are excluded because they represent structurally different data-exchange mechanisms and would otherwise dominate any comparison by volume alone.

2. Data and Methodology

2.1 Source and Screening

Source: Clarity detail-level records (DOCS_SENT, DOCS_SENT_DETAILS, DOCS_SENT_AUDIT), joined to ORG_DETAILS, ZC_DISP_ENC_TYPE, ZC_DOC_CONTENT_TYPE, PAT_ENC, PATIENT, and PATIENT_RACE. This is a single, row-level source used consistently for both the volume/statistical analysis and the encounter-type/demographic layer, replacing the two separately-sourced datasets used in the prior revision.

Of the 432.6 million rows under organization type 1, 179,069,316 (41.4%) fall within the report window (July 1, 2023 - June 30, 2026) with a valid completion record. The remainder is split between pre-window history (22.0%), post-window records (0.8%), and requests with no completion timestamp or audit record at all (35.8%) - the latter representing incomplete or pending requests, excluded from this analysis by design rather than by omission.

Table 1. Data screening summary, organization type 1, all dates.

Category	Rows	% of Total
Inside report window, completed	179,069,316	41.4%
No completion timestamp / no audit record	154,701,012	35.8%
Before July 1, 2023	95,202,278	22.0%
After June 30, 2026	3,629,438	0.8%

2.2 Statistical Methods

- Cross-sectional z-score: for every organization on every day, $z = (\text{organization's pull count} - \text{mean of all other organizations that day}) / \text{standard deviation of all other organizations that day}$, computed at daily grain. A conventional outlier threshold is $|z| > 3$; every organization discussed in this report exceeds that threshold by at least an order of magnitude.
- Growth ratio: each organization's average daily volume in a recent window (January - June 2026) compared to an early baseline window (October 2023 - March 2024).
- Completed pulls only: filtered to REQ_STATUS_C = 3 with a valid send timestamp, confirmed by direct diagnostic to represent genuinely completed document sends (93.3% of Company 1's total volume).

2.3 A Note on the Prior Revision

The original version of this analysis used a pre-aggregated, daily-grain export limited in scope to basic statistics identified through an initial manual review. This revision replaces that source entirely with the row-level extract described above, recomputes every original finding, and extends the same tests to all connected organizations. Findings that changed materially between revisions are noted explicitly in the relevant section below rather than silently updated.

3. Results

3.1 Network-Wide Comparative Ranking

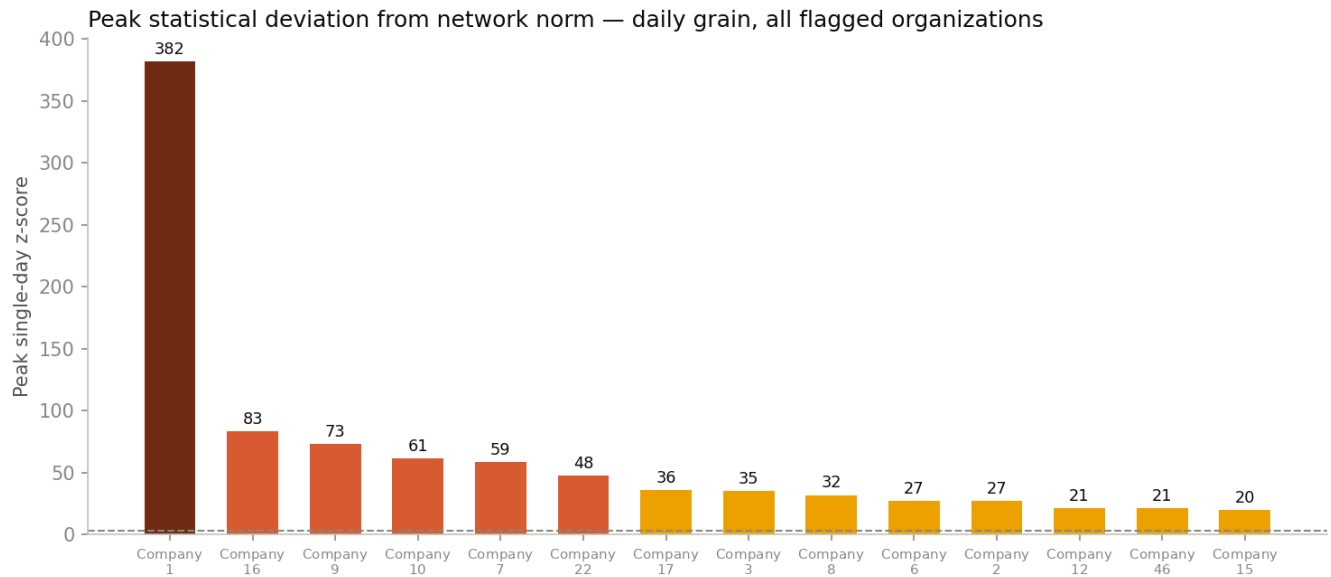


Figure 1. Peak single-day z-score, daily grain, all organizations discussed in this report.

Organizations above the conventional $|z| = 3$ threshold are, by definition, statistical outliers. Every organization named in this report exceeds that threshold by at least an order of magnitude; several exceed it by more than one hundred times. The pattern each organization follows - sustained, episodic, lifecycle, erratic, or scale-driven - determines the appropriate response, which is why findings are grouped by pattern below rather than by z-score alone.

3.2 Tier 1: Active, Ongoing Surges

3.2.1: Company 1

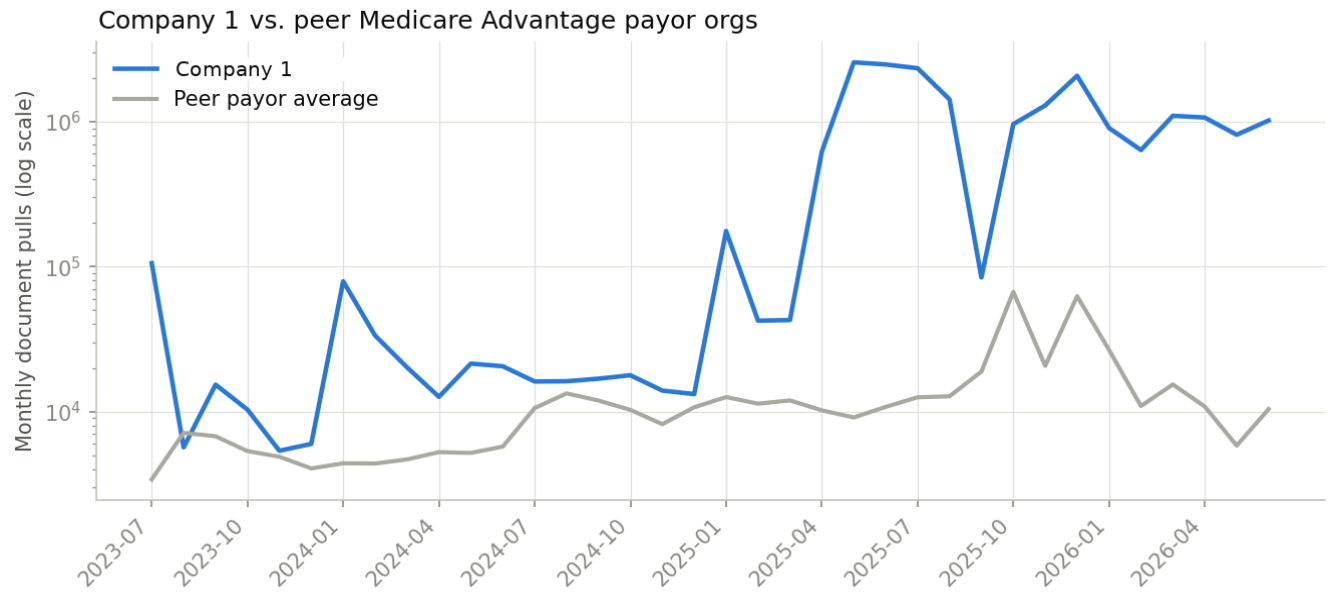


Figure 2. Company 1 vs. peer Medicare Advantage payor organizations, monthly pull volume, log scale.

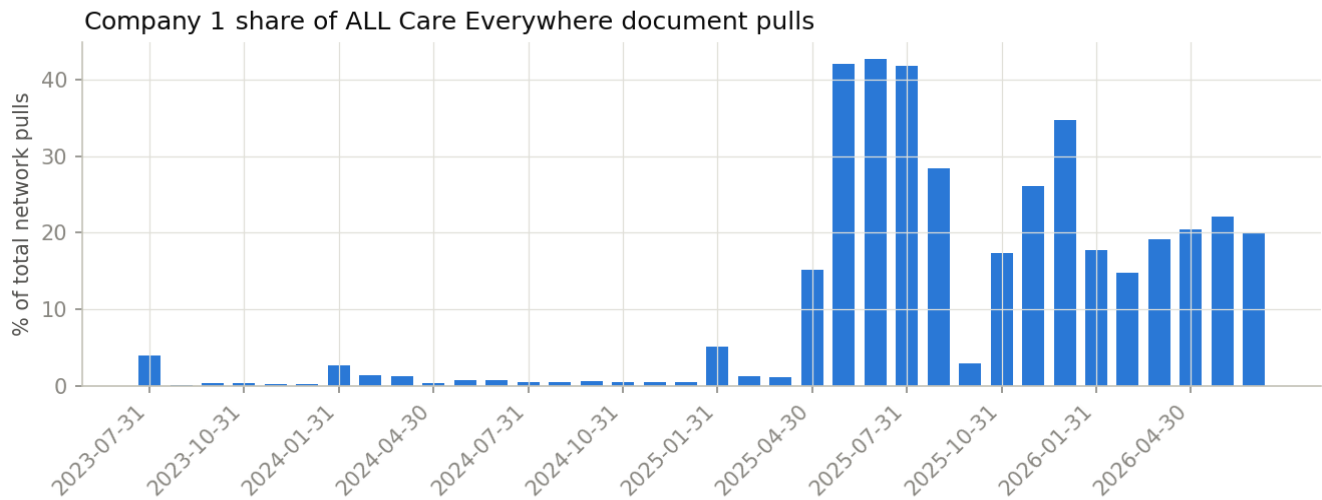


Figure 3. Company 1's share of all Care Everywhere document pulls network-wide, by month.

Original findings (monthly grain, nine-organization screen):

Baseline monthly volume (Oct 2023 - Mar 2024): approximately 5,000-20,000 pulls/month

Peak monthly volume (May 2025): 2.57 million pulls

Peak share of ALL network pulls (June 2025): 42.7%

Current share of network pulls (June 2026): approximately 20%

Confirmed and refined by this revision (daily grain, all-organization screen):

Baseline average (October 2023 - March 2024): approximately 1,024 pulls/day

Recent average (January - June 2026): approximately 32,423 pulls/day

Growth ratio: 31.6x

Peak single-day z-score (December 20, 2025): $z = 381.9$

Mean z-score across 1,058 active days: $z = 9.8$

The surge is a step-change beginning April 2025 that has not reverted. Recomputation at daily grain confirms the original finding with substantially greater precision - the peak day now identified reaches a z-score 26x higher than the original monthly-grain estimate, reflecting the sharper resolution of daily data rather than a change in the underlying pattern.

3.2.2 Company 2

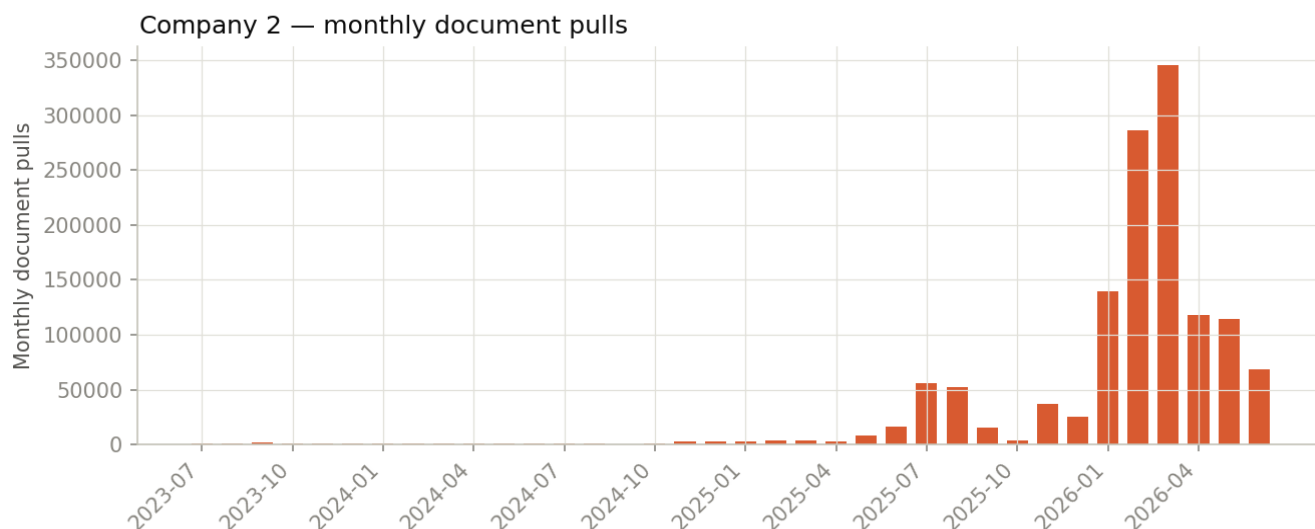


Figure 4. Company 2, monthly document pulls.

Original findings:

Baseline monthly volume (Oct 2023 - Mar 2024): approximately 1,263 pulls/month

Peak monthly volume (March 2026): 345,427 pulls

Original growth ratio (monthly baseline vs. recent): 79.4x - highest of any organization in the original review

Confirmed by this revision:

Growth ratio (daily baseline vs. recent): 73.3x - still the highest sustained-surge ratio of any organization reviewed

Peak single-day z-score: $z = 27.0$

A single specialty medical practice reaching pull volumes comparable to major regional health systems remains inconsistent with its organization type.

3.2.3 Company 3 and Related Connections

Newly identified rapid-growth connections, monthly pulls

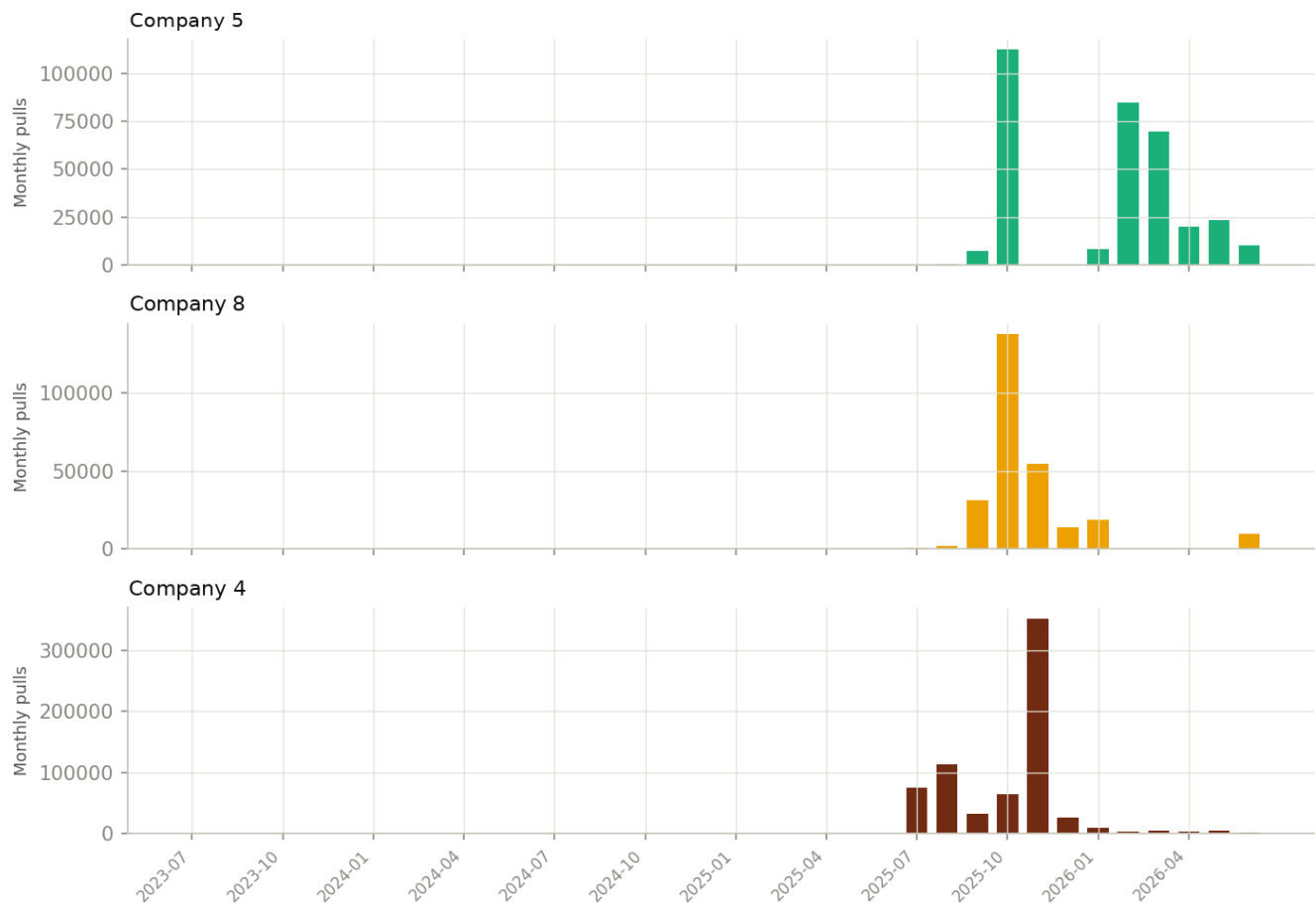


Figure 5. Newly identified rapid-growth connections, monthly pulls: Company 5, Company 8, and Company 4.

Original findings:

Peak monthly volume (March 2026): 345,395 pulls

Current run-rate (Apr-Jun 2026 average): approximately 125,500 pulls/month

Confirmed by this revision:

Growth ratio: effectively unbounded (zero baseline; approximately 5,031 pulls/day recent average)

Peak single-day z-score: $z = 35.3$

Company 4, a Company 3-platform-affiliated connection, reached a single-month peak of 352,270 pulls in November 2025 before declining sharply toward zero by mid-2026, consistent with a completed onboarding burst rather than an ongoing surge. Company 5, a genetics and diagnostics laboratory with no prior connection history, is shown in the same figure as a separate new finding of real, growing volume with no antecedent baseline.

3.3 Tier 2: Resolved, Episodic Anomalies

3.3.1 Company 6

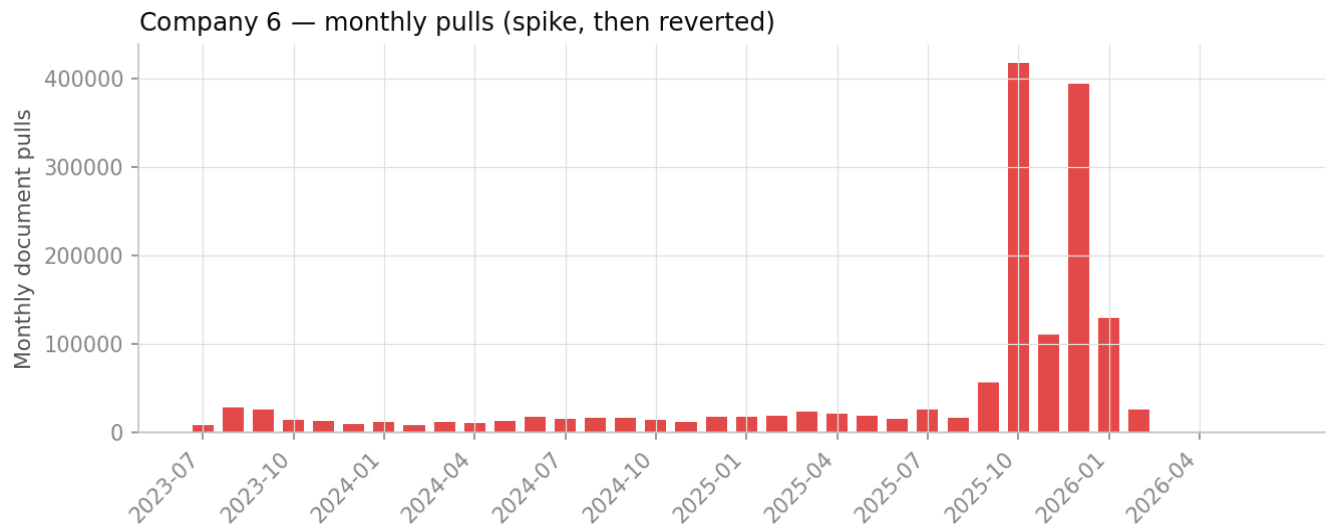


Figure 6. Company 6, monthly document pulls.

Original findings:

Stable baseline (2023-2025): approximately 8,000-27,000 pulls/month

Spike peak (October 2025): 418,049 pulls

Current volume (April-June 2026): under 250 pulls/month - effectively inactive

Original peak single-day z-score: $z = 41.8$

Confirmed by this revision:

Peak single-day z-score (daily grain): $z = 27.1$

A five-month spike (September 2025 - February 2026) followed by a near-total drop to under 250 pulls per month - the original finding stands, with the volume above and below the spike unchanged by this revision's recomputation.

3.3.2 Company 7 and Company 8

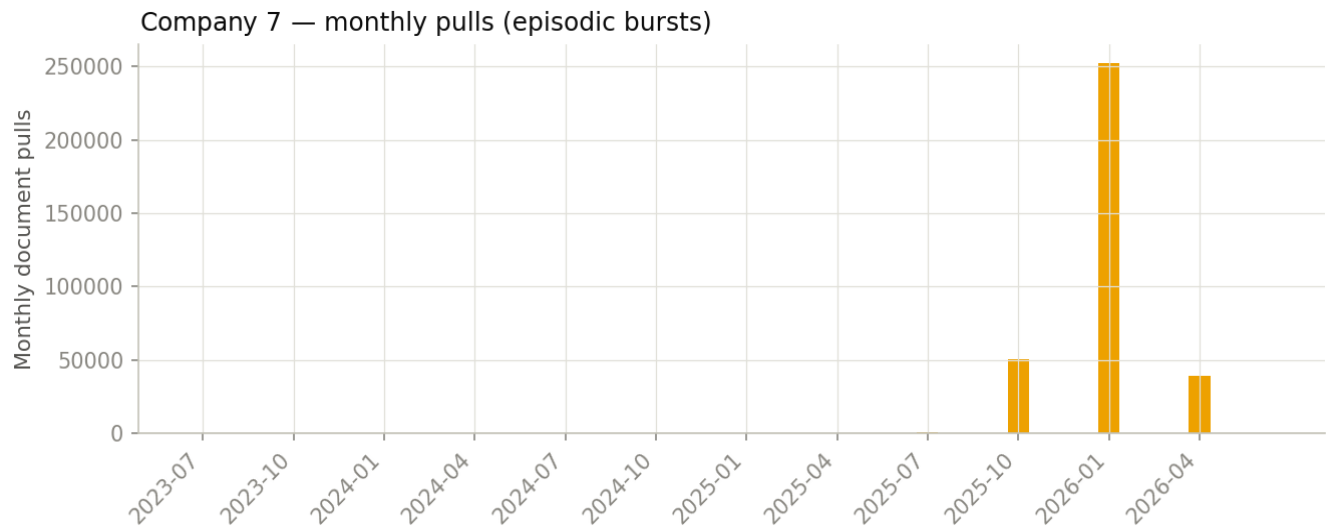


Figure 7. Company 7, monthly document pulls (episodic bursts).

Original findings - active months (all other months at or near zero):

Table 2. Company 7, isolated high-volume months.

Month	Pulls
April 2025	395
July 2025	1,038
October 2025	50,784
January 2026	252,498
April 2026	39,335

Original peak single-day z-score (January 2026): $z = 97.9$

Confirmed by this revision:

Company 7 peak single-day z-score (daily grain): $z = 58.7$

Company 8 peak single-day z-score: $z = 31.8$

Company 8, a related connection with no history before February 2025, peaked at 137,595 pulls in October 2025 before declining - the same bursty signature as Company 7 itself, reinforcing the original finding rather than contradicting it. The isolated-month pattern in the table above is the statistical signature of a manual or scheduled batch export rather than day-to-day clinical document retrieval.

3.4 Tier 3: Connection Lifecycle Events

3.4.1 Historical Brief Connections

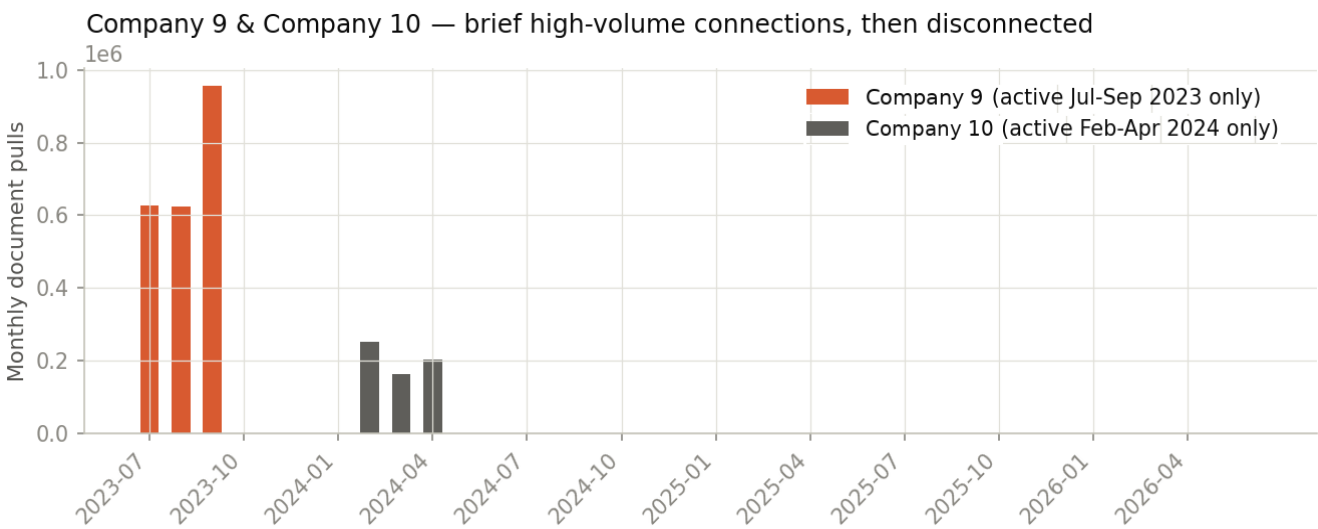


Figure 8. Company 9 and Company 10, brief high-volume windows followed by disconnection.

Original findings:

Company 9: 628K-956K pulls/month, active July-September 2023 only; zero every month since

Company 10: 163K-253K pulls/month, active February-April 2024 only; zero every month since

Confirmed by this revision:

Company 9 peak single-day z-score: $z = 73.0$

Company 10 peak single-day z-score: $z = 61.4$

3.4.2 Newly identified: recently discontinued connections.

Extending the screen to all organizations surfaced three additional connections not part of the original review, each showing substantial historical volume that dropped to effectively zero by the end of the report window - the inverse of the onboarding-burst pattern above.

Table 3. Recently discontinued connections, baseline vs. recent average daily volume.

Organization	Baseline Avg/Day	Recent Avg/Day	Ratio	Notes
Company 11	1,894	0	0.0005x	Digital cardiac monitoring platform
Company 12	5,056	0	0.0002x	Regional health information network
Company 13	5,253	0	0.0002x	Public health reporting feed

3.5 Tier 4: Erratic, High-Volatility Organizations (Newly Identified)

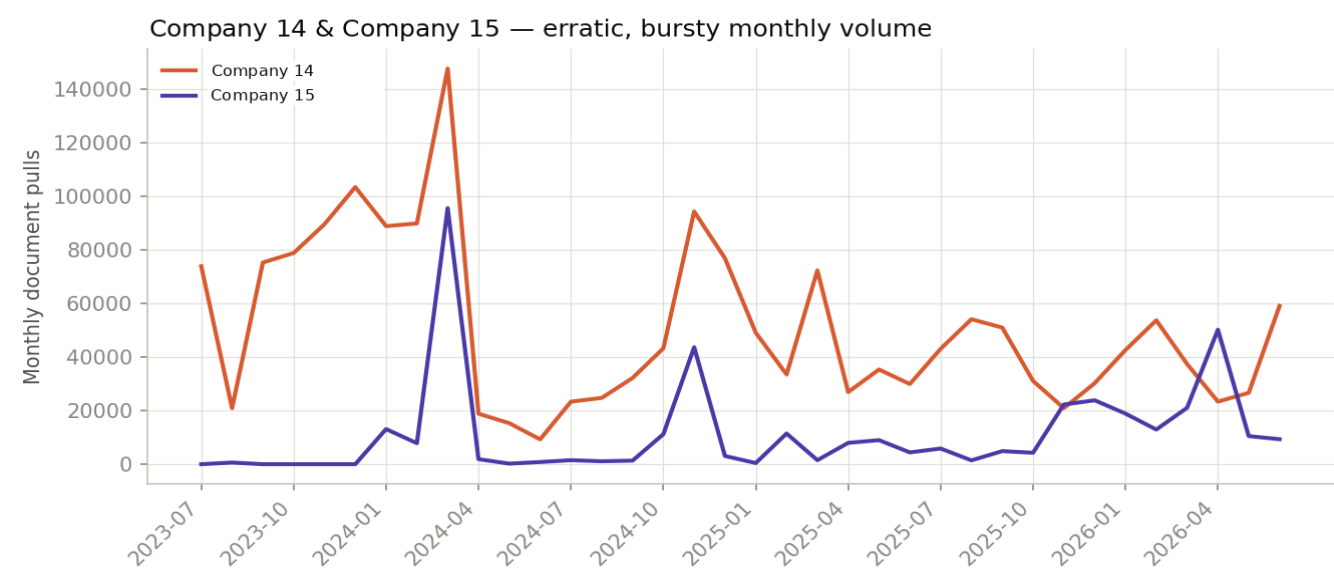


Figure 9. Company 14 and Company 15, monthly pull volume - irregular, bursty, no clear secular trend.

Company 14, month-to-month range: 9,278 to 147,872 pulls
Company 15, month-to-month range: 207 to 95,693 pulls

Both organizations were initially flagged by the growth-ratio screen as "declining"; closer inspection of the full monthly series shows an erratic, recurring-but-irregular pattern rather than a smooth decline. Neither shows the sustained step-change of Tier 1 nor the clean single-window spike of Tier 2, perhaps indicative of quarterly or semi-regular organizational data pulls.

3.6 Tier 5: Large-Scale, Non-Anomalous Organizations

3.6.1 Company 16 (newly identified)

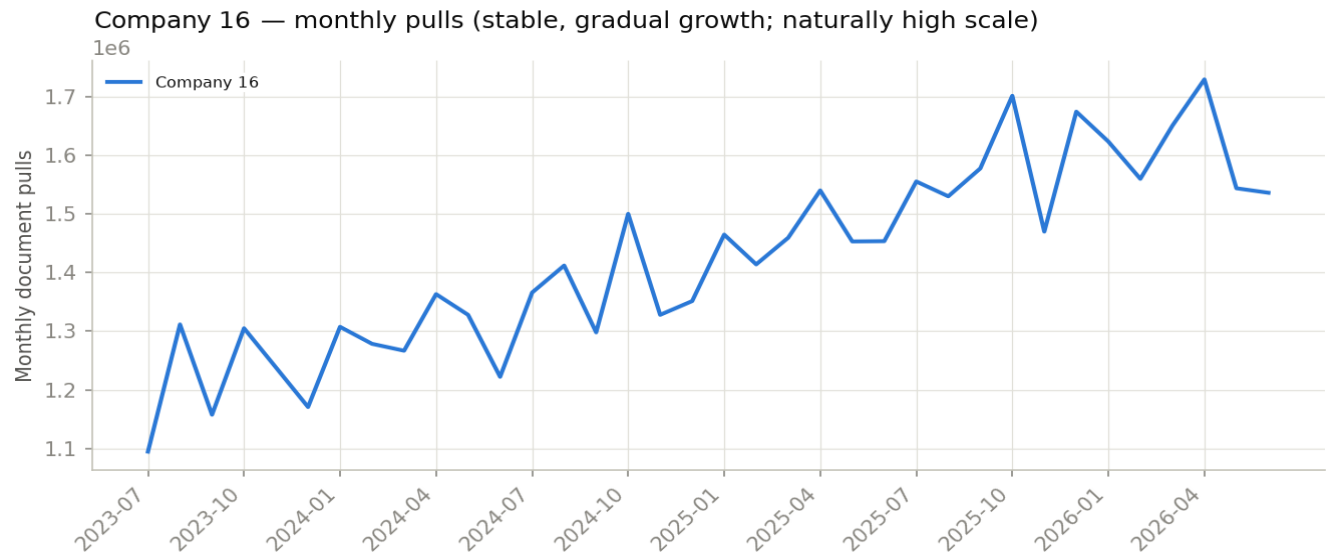


Figure 10. Company 16, monthly document pulls - stable, gradual growth.

Growth ratio: 1.29x - stable, gradual increase, no step-change

Mean daily z-score across 1,096 days: $z = 24.2$

Peak single-day z-score: $z = 83.0$

Share of pulls with no encounter type: 91.0%

Company 16's volume grew smoothly from approximately 1.1 million to 1.7 million pulls per month over three years. Its high z-score is a direct mathematical consequence of its scale relative to a peer population of mostly much smaller organizations, not evidence of anomalous behavior. It is reported here as a scale-driven outlier, distinct in kind from the Tier 1 surge organizations.

3.6.2 Company 17 and Other Large, Stable Organizations

Original findings, Company 17:

2023-2024 baseline: approximately 501,000 pulls/month

2026 current run-rate: approximately 1,089,000 pulls/month (+117%)

Table 4. Large organizations confirmed stable across the report window.

Organization	Baseline Avg/Day	Recent Avg/Day	Growth Ratio
Company 17	16,701	36,300	2.2x - consistent with known federal HIE expansion
Company 18	1,826	2,325	1.3x
Company 19	3,226	3,917	1.2x

The single largest connection in the dataset by total volume, Company 17's growth is gradual and multi-year, consistent with known, ongoing federal health-information-exchange modernization efforts rather than an anomaly.

3.6.3 Company 22

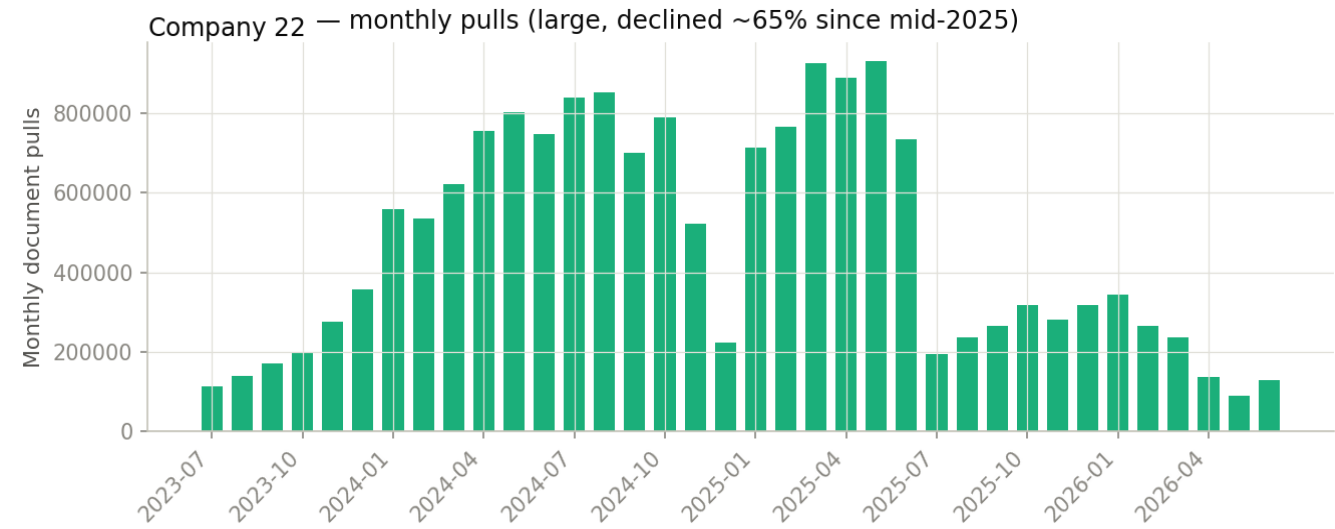


Figure 11. Company 22, monthly document pulls - large, declined approximately 65% since mid-2025.

Peak monthly volume (May 2025): 931,717 pulls
Current volume (June 2026): 128,662 pulls (-72% vs. baseline)
Peak single-day z-score (daily grain): $z = 47.7$

A sustained large-scale drop-off is as worth investigating as a spike, since it may indicate a lost data-sharing relationship, a technical integration problem, or activity shifting to a different organization ID. This finding is unchanged by the revision and remains one of the more significant unresolved findings in this report and should not be read as lower-priority simply because it involves a decline rather than a spike.

3.7 Other Organizations Outside Expected Values

The following organizations show growth ratios of 3x or greater against a meaningful baseline, but their absolute scale keeps their network-level z-scores low - elevated relative to their own history, but not statistically abnormal relative to the network as a whole. Retained from the original analysis for completeness.

Table 5. Organizations with elevated growth ratios but low absolute z-scores.

Organization	Growth Ratio	Likely Context
Company 23	20.6x	Genetic testing lab - referral/testing volume growth
Company 24	6.2x	Medicare Advantage-affiliated primary care expansion
Company 25	6.4x	Large health system - real absolute increase given size
Company 5 (secondary listing)	17.2x	See Tier 1, Section 3.2.3 for primary discussion
Company 26	20.3x	Value-based kidney care - plausible panel growth
Company 27	8.2x	Telehealth dietitian network - platform growth
Company 28	4.4x	Dialysis provider - plausible panel growth
Company 29	35.1x	Small scale; timing coincides with Company 2
Company 30	47.3x	Small scale; newer connection
Company 31	46.6x	Small scale; steady incremental climb

4. Methodology Correction: The "Percent Unspecified Encounter" Signal

The prior revision of this report treated a high share of pulls with no attached encounter type as suggestive evidence of bulk or non-clinical activity, based on Tier 1 organizations showing elevated rates relative to a nine-organization comparison set. Extending the same measurement to every organization in the network overturns that interpretation.

Table 6. Share of pulls with no attached encounter type, selected organizations network-wide.

Organization	% Pulls with No Encounter Type
Company 32	100.0%
Company 21	98.4%
Company 20 Southern California	98.3%
Company 33	98.1%
Company 16	91.0%
Company 1	66.1%
Company 2	51.9%

Several of the largest, most established health systems nationally show unspecified-encounter rates well above Company 1's. This indicates the measurement reflects something structural about how document exchange is tied to encounters in Clarity, generally, rather than a specific signature of automated or non-clinical activity. This report retains the metric only as a within-organization time-series check against known event timing, demonstrated below, and no longer as a cross-organization comparison.

5. Time-Correlation Analysis

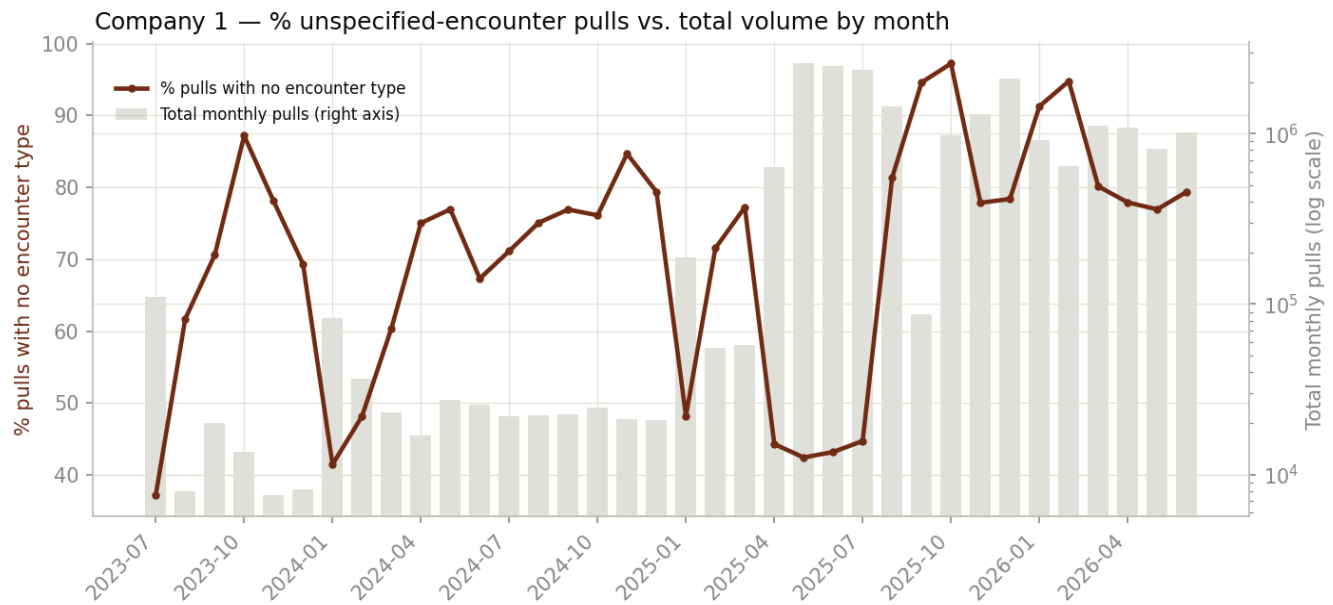


Figure 12. Company 1, monthly percent of pulls with no encounter type, plotted against total monthly volume.

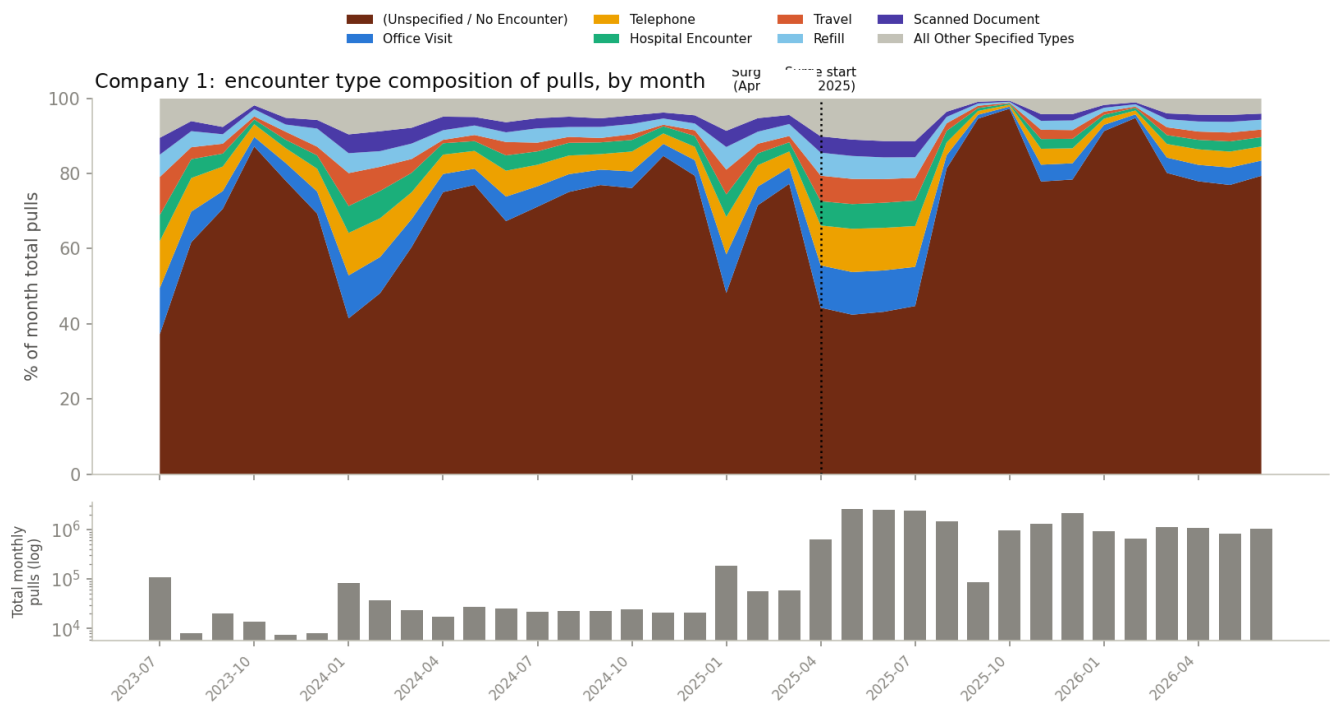


Figure 13. Company 1's monthly document pulls by encounter type, July 2023–June 2026, showing the mix shift from roughly 68% unspecified before the surge, down to 44% during the surge's sharpest months (April–July 2025), then up to 85% afterward.

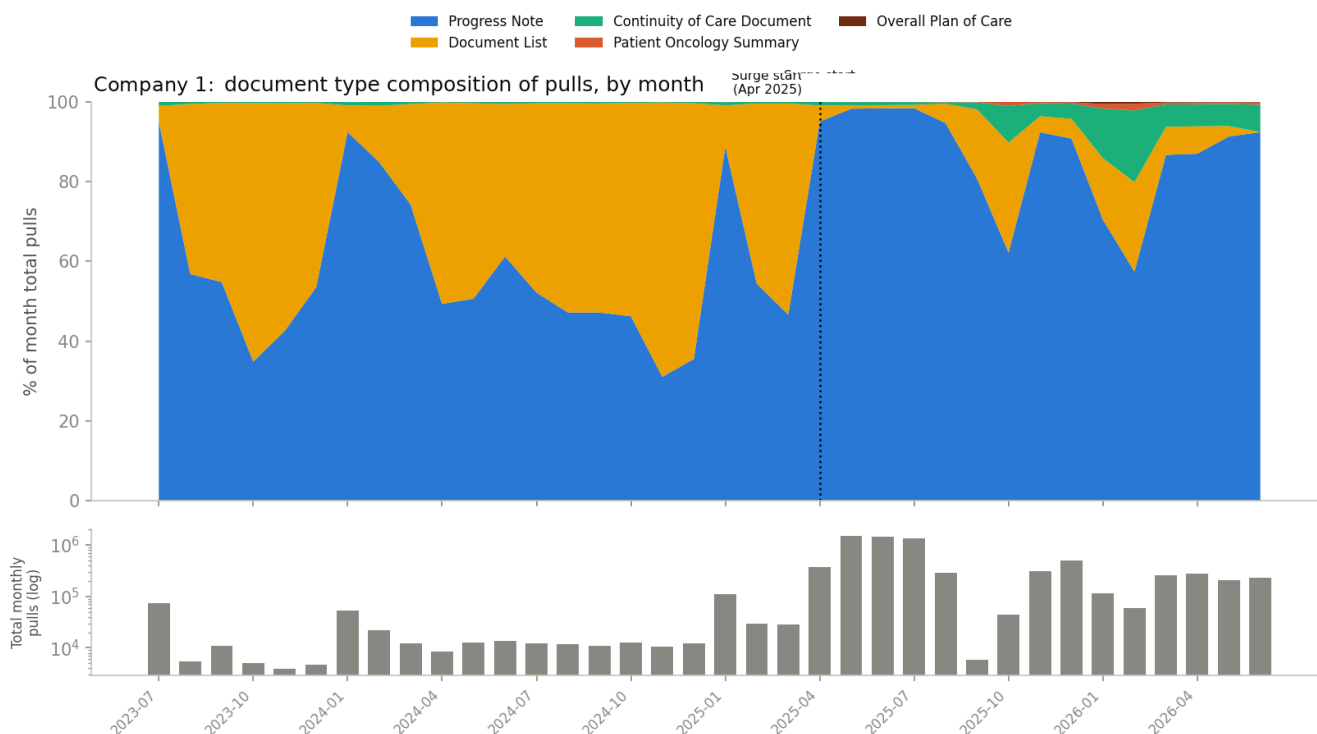


Figure 14. Company 1's monthly document pulls by document type, July 2023-June 2026, showing a shift from a roughly even Progress Note/Document List split before the surge to 97.5% Progress Note during the surge's sharpest months (April-July 2025).

This test is more rigorous than the static cross-organization comparison above, and it complicates rather than confirms the original hypothesis. Company 1's monthly unspecified-encounter rate was already elevated (60-87%) well before the April 2025 volume surge began. During the surge's most extreme months (April - July 2025), the rate dropped to its lowest point in the dataset (42-45%), meaning the additional volume during the sharpest part of the surge was, if anything, more likely to carry a real encounter type. The rate returned to 90%+ later (September 2025 - February 2026), well after the initial jump. This does not rule out an automated or administrative process driving the surge, but it does rule out the simplest version of that hypothesis for the initial surge specifically. The later-period rise merits its own follow-up.

The encounter-type composition itself, not just its overall rate, sharpens this picture further. Before the surge, Company 1's unspecified pulls sat alongside a real mix of specified encounter types with Office Visit, Telephone, Hospital Encounter, Travel, Refill, and Scanned Document each carrying a meaningful share. During the surge's sharpest months, every one of these specified categories roughly doubled its share of monthly volume (for example, Office Visit rose from 6.1% to 11.0%, Telephone from 6.3% to 11.1%), which is consistent with the drop in the unspecified rate noted above rather than a separate finding. In the post-surge period, the pattern reverses beyond its starting point: unspecified climbs to 84.5%, higher than the pre-surge baseline, while every specified category falls to roughly half its earlier share - indicating the later-period rise in unspecified pulls is broad-based across encounter types, not concentrated in any single category.

Document type data points to a plausible mechanical contributor to this pattern. Before the surge, Company 1's pulls were split roughly evenly between Progress Note (57.1%) and Document List (42.4%). During the surge's sharpest months, this shifts almost entirely to Progress Note (97.5%), with Document List nearly disappearing (1.6%). Assuming Progress Notes are generated as standalone clinical documentation not always tied to a scheduled, codeable encounter while a Document List is generated alongside a specific visit, a surge concentrated almost entirely in Progress Notes is consistent with, and may partly explain, the same surge's disproportionately low encounter-type attachment. This is offered as a plausible contributing mechanism, not a confirmed cause, and does not by itself explain the later-period unspecified-rate rise described above.

6. Demographic Findings

Demographic data exists only for pulls tied to a specific encounter; the unspecified-encounter share reported in Section 4 carries no demographic data by definition. The findings below describe only the subset of each organization's volume where a demographic link exists and should not be read as representative of total volume. Coverage varies substantially by organization - from 6% of total volume for Company 16 to nearly 100% for Company 13 - and that coverage rate is reported alongside each finding.

6.1 Age Distribution

Age bands are computed from patient date of birth as of the pull date, among pulls with a known encounter link. Organizations vary widely - a genuine finding in its own right, not noise to be averaged away.

Table 7. Age distribution summary, known-age pulls only, all flagged organizations.

Organization	Known-Age Volume	% Aged 65+	% Aged Under 35
Company 7	202,879	97.3%	0.2%
Company 15	238,208	87.7%	0.3%
Company 1	6,910,086	84.4%	0.1%
Company 9	1,508,122	79.5%	0.6%
Company 10	600,164	78.4%	1.3%
Company 6	1,023,993	75.9%	0.3%
Company 3	380,249	75.0%	0.5%
Company 22	16,487,458	72.9%	1.0%
Company 4	348,841	72.1%	0.2%
Company 17	22,892,097	71.3%	2.2%
Company 14	1,036,048	64.0%	0.1%
Company 12	106,566	51.0%	4.2%
Company 2	708,898	48.7%	19.1%
Company 5	193,378	47.8%	4.4%
Company 16	4,600,797	38.5%	25.4%
Company 13	3,488,591	31.1%	37.0%
Company 11	331,037	16.4%	2.9%

The single most notable finding in this table is Company 7: 97.3% of its known-age pulls are for patients aged 65 or older. Combined with the burst, all-or-nothing volume pattern documented in Section 3.3.2, an elderly-skewed population pulled in large irregular batches is consistent with a value-based senior care chart-retrieval process, similar in kind to the Company 6 finding, though this remains a hypothesis rather than a confirmed cause.

Company 1 (84.4% 65+) and Company 15 (87.7% 65+) are consistent with their expected populations - a Medicare Advantage payor and a value-based chronic-care management organization, respectively. Company 16 (38.5% 65+, 25.4% under 35) and Company 13 (31.1% 65+, 37.0% under 35) both show broad, general-population distributions consistent with a comprehensive regional health system and an all-ages public health reporting feed. Company 11's young skew (16.4% 65+) is consistent with a consumer digital health platform.

6.2 Race Distribution

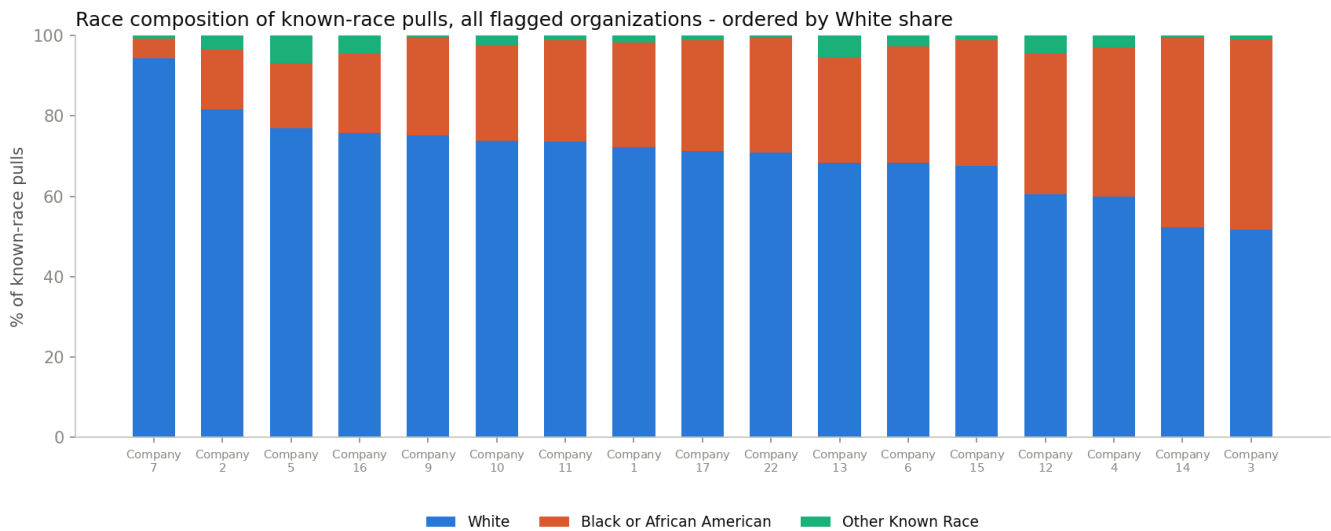


Figure 15. Race composition (White vs. Black or African American share) of known-race pulls, all flagged organizations.

Most organizations in this analysis cluster in a 68-76% White / 20-30% Black or African American range among known-race pulls - broadly consistent with general regional demographics for the State 1 service area. Three organizations fall clearly outside that range in opposite directions and are flagged here for that reason alone; no cause is inferred from the pattern itself.

- Company 3 (51.7% White / 47.2% Black) and Company 14 (52.3% White / 47.2% Black) both show a near-even split, substantially more Black representation than the network-wide pattern. The two organizations' figures are close enough to each other that a shared underlying patient population or referral source is worth checking directly, rather than treating them as two unrelated findings.
- Company 7 (94.1% White / 5.0% Black) shows the least racial diversity of any organization reviewed - the inverse pattern from Company 3 and Company 14, and worth reading alongside its age-distribution finding above rather than in isolation.

None of these patterns are, on their own, evidence of a specific cause - a legitimate explanation (a platform's actual served population, a referral network's geographic footprint, a specific plan's enrollment demographics) is at least as plausible as a data-quality or process concern. They are reported because a pattern this consistent across two related-in-timing organizations (Company 3, Company 14) and this extreme in the opposite direction for another already-flagged organization (Company 7) meets the same bar for disclosure as the volume and z-score findings elsewhere in this report.

6.3 Statistical Test: Age Distribution, Five Largest Organizations vs. Network Baseline

The descriptive comparisons above show that distributions differ; they do not establish how much they differ or which specific age bands drive the difference. This section applies a chi-square goodness-of-fit test to each of the five largest organizations by total volume (Company 16, Company 17, Company 1, Company 22, and Company 19), comparing each organization's observed age-band distribution against an expected distribution built from every other organization in the network (a leave-one-out baseline, so no organization's own volume inflates the standard it is being measured against). Cramer's V is reported as the effect size, since chi-square significance alone is close to guaranteed at this sample size and is not informative on its own; standardized residuals identify which specific age bands are over- or under-represented.

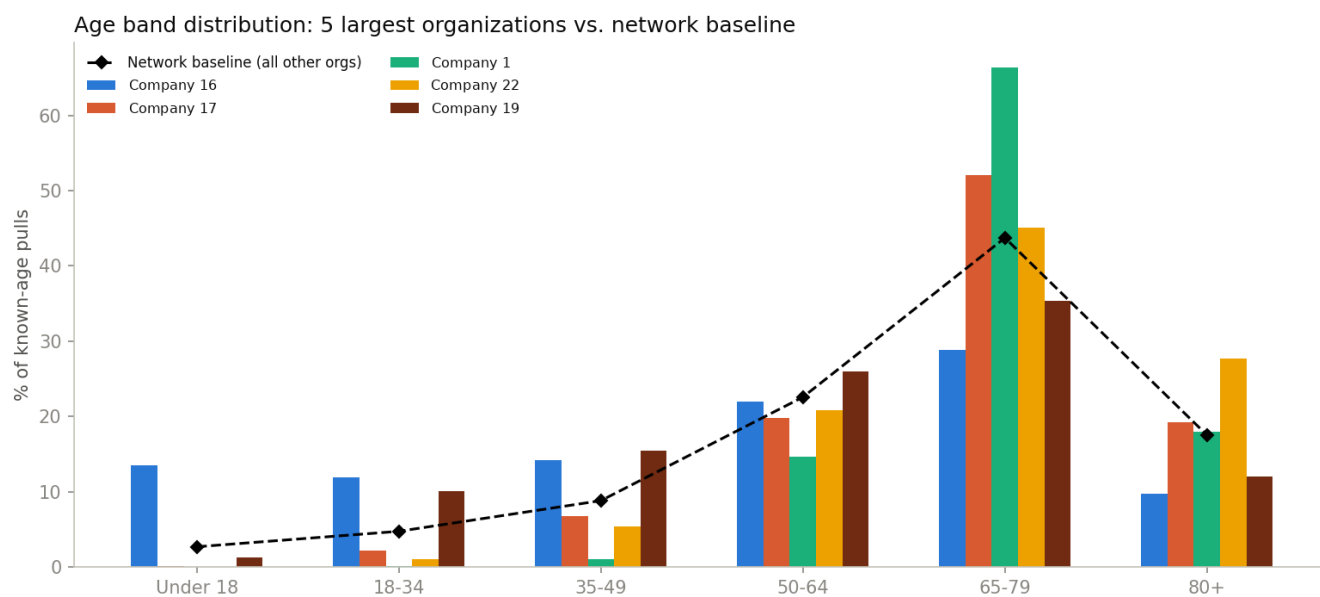


Figure 16. Age band distribution, five largest organizations vs. network baseline (all other organizations).

Table 8. Chi-square goodness-of-fit test results, age band distribution, five largest organizations vs. network baseline.

Organization	N (known-age)	Chi-Square (df=5)	Cramer's V	Most Over-Represented	Most Under-Represented
Company 16	4,600,797	3,971,297	0.415 (large)	Under 18 (13.4% vs. 2.1% baseline)	65-79 (28.8% vs. 44.5% baseline)
Company 1	6,910,086	2,260,837	0.256 (medium)	65-79 (66.4% vs. 42.0% baseline)	35-49 (1.0% vs. 9.4% baseline)
Company 22	16,487,458	3,106,224	0.194 (small-medium)	80+ (27.7% vs. 15.4% baseline)	18-34 (1.0% vs. 5.5% baseline)
Company 19	1,069,007	172,966	0.180 (small-medium)	18-34 (10.1% vs. 4.7% baseline)	65-79 (35.3% vs. 43.8% baseline)
Company 17	22,892,097	2,290,114	0.141 (small)	65-79 (52.1% vs. 41.1% baseline)	Under 18 (0.1% vs. 3.5% baseline)

All five organizations differ from the network baseline at $p < 0.001$, but the effect sizes vary meaningfully and each tells a different, individually plausible story.

Company 16 shows the largest effect (Cramer's $V = 0.415$): it substantially over-represents younger patients (Under 18 and 18-34 combined account for 25.4% of its known-age volume versus 6.5% network-wide) and under-represents the 65-79 band - consistent with its role as a comprehensive regional health system serving a full-age population, in contrast to a network otherwise dominated by senior-focused payors and programs.

Company 1 shows the next-largest effect ($V = 0.256$), concentrated entirely in a strong over-representation of the 65-79 band, consistent with its identity as a Medicare Advantage payor.

Company 22's largest deviation is specifically in the 80+ band (27.7% vs. 15.4% baseline) - a stronger skew toward the oldest patients than any other organization in this test, worth noting given this organization's separate, unresolved volume-decline finding in Section 3.6.3.

Company 17's pattern ($V = 0.141$, the smallest effect among the five) is consistent with a veteran population skewing older with very little pediatric volume. None of these five patterns, taken on their own, indicate an implausible or unexplained population; each is consistent with the organization's known role.

6.4 Statistical Test: Race Distribution, Five Largest Organizations vs. Network Baseline

The same test, applied to race. Race categories are collapsed to White, Black or African American, and a combined Other Known Race category (all remaining specific race categories, which are individually small); records with no race recorded, marked Unknown, or Decline to Answer are excluded from this test rather than folded into Other, since they represent absence of data rather than a race category.

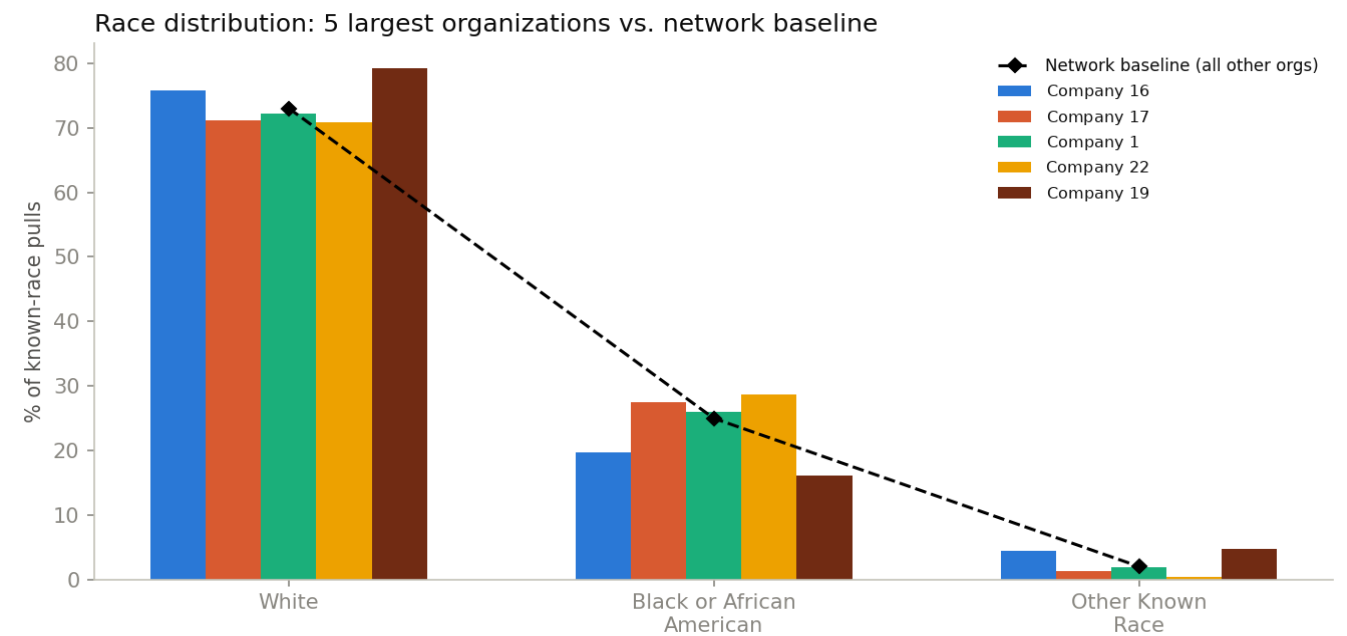


Figure 17. Race distribution, five largest organizations vs. network baseline (all other organizations).

Table 9. Chi-square goodness-of-fit test results, race distribution, five largest organizations vs. network baseline.

Organization	N (known-race)	Chi-Square (df=2)	Cramer's V	Most Over-Represented	Most Under-Represented
Company 19	1,052,753	79,255	0.194 (small-medium)	Other Known Race (4.7% vs. 2.0% baseline)	Black or African American (16.1% vs. 25.1% baseline)
Company 16	4,545,978	217,480	0.155 (small)	Other Known Race (4.5% vs. 1.9% baseline)	Black or African American (19.7% vs. 25.2% baseline)
Company 22	16,421,114	407,161	0.111 (small)	Black or African American (28.7% vs. 24.2% baseline)	Other Known Race (0.4% vs. 2.3% baseline)
Company 17	22,752,396	224,362	0.070 (small)	Black or African American (27.6% vs. 24.2% baseline)	Other Known Race (1.3% vs. 2.3% baseline)
Company 1	6,876,887	4,594	0.018 (negligible)	Black or African American (25.9% vs. 24.9% baseline)	Other Known Race (1.9% vs. 2.0% baseline)

Race-distribution effects among the five largest organizations are consistently smaller than the age effects above - the largest, Company 19 at $V = 0.194$, is still only "small-to-medium" by conventional effect-size guidance, and Company 1's deviation ($V = 0.018$) is negligible - its known-race distribution is close to indistinguishable from the network baseline. This is a meaningful finding in its own right: whatever is driving Company 1's extreme volume surge (Section 3.2.1), it is not associated with a detectable shift in which patients, by race, are having their records pulled. Company 19 and Company 16 both under-represent Black patients and over-represent the small Other Known Race category relative to baseline; Company

22 and Company 17 both run in the opposite direction, over-representing Black patients slightly. None of these five effects are large by conventional standards, in contrast to the age-distribution test above where two organizations (Company 16, Company 1) showed medium-to-large effects.

Read together, Sections 6.3 and 6.4 support a specific, bounded conclusion: the five largest organizations in this network show real, statistically detectable differences in the age of patients whose records are pulled, and those differences are individually explainable by each organization's known role. Race-based differences are present but consistently weaker, and in Company 1's specific case - the organization carrying this report's most serious volume finding - are effectively absent. This does not rule out partiality by race or age at a level too small for this test to detect, and does not extend to organizations outside the five tested here (several of which, per Section 6.2, show larger race deviations than any of the top five). It does mean that, for the organizations carrying the most total volume in this network, age and race patterns do not independently corroborate a concern beyond what the volume and z-score findings elsewhere in this report already establish.

6.5 Age and Race Composition Over Time

The tests above are static - they compare each organization's demographic mix, pooled across the full report window, against a single baseline. This section instead tracks each organization's age and race composition month by month, as a percent of that organization's own known-demographic volume, to test directly whether composition shifts alongside the volume changes documented in Section 3 - the more informative question when a specific organization's volume itself is under scrutiny.

Company 1's age composition is the most important result in this section: its share of pulls aged 65 and older stays within a 75-92% band for the entire three-year window, including the exact months surrounding the April 2025 volume surge (March 2025: 87.5%; April 2025: 83.5%; May 2025: 86.1%). The surge added volume, but did not shift who that volume was about - the additional pulls were proportionally consistent with Company 1's existing population, not concentrated in a different age group.

Company 16 shows the same pattern of stability, holding within a narrow 34-40% 65-plus band across all 36 months with no shift around any of its growth. Company 22 is the one organization in this group where composition moves: its 65-plus share dipped as low as 59.1% (December 2024) and 66.0% (August 2025), then climbed to a three-year high of 79-80% across January-March 2026 - a period during which its total volume was already in decline (Section 3.6.3).

That combination of falling volume and rising elderly share is consistent with younger or lower-priority patient categories dropping out of an already-shrinking pull stream faster than older ones, though this is an observation to investigate, not a conclusion this analysis can confirm on its own.

Race composition over time is stable for all five organizations, without exception; no organization shows a visible shift in its White / Black or African American / Other Known Race proportions coinciding with any volume event documented elsewhere in this report, including Company 1's surge and Company 22's decline. This reinforces the static finding in Section 6.4: race composition does not appear to be a factor in any of the volume changes under investigation for these five organizations.

Age band composition over time, five largest organizations (% of known-age pulls, monthly)

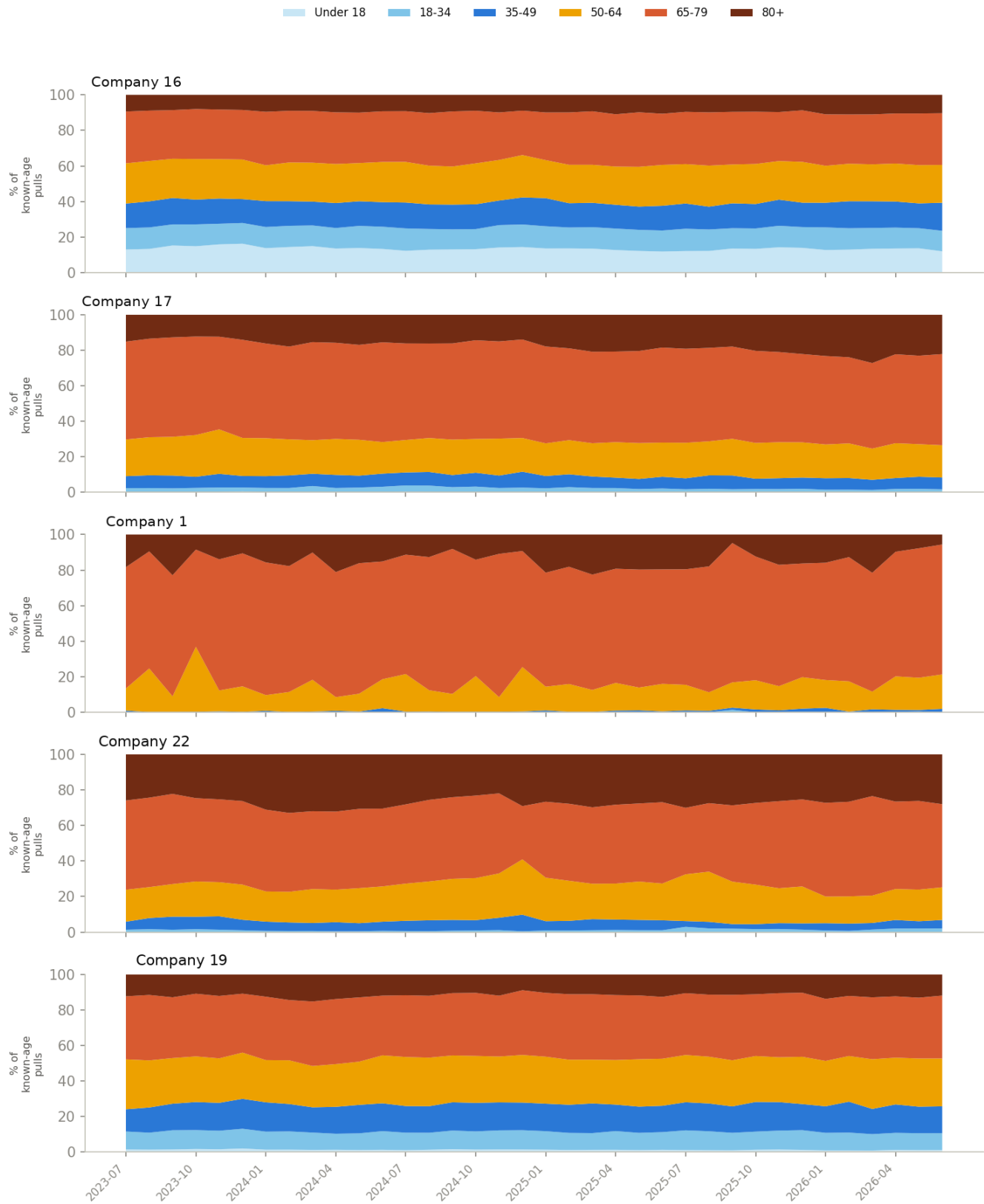


Figure 18. Age band composition over time, five largest organizations, as a percent of known-age pulls each month.

Race composition over time, five largest organizations (% of known-race pulls, monthly)

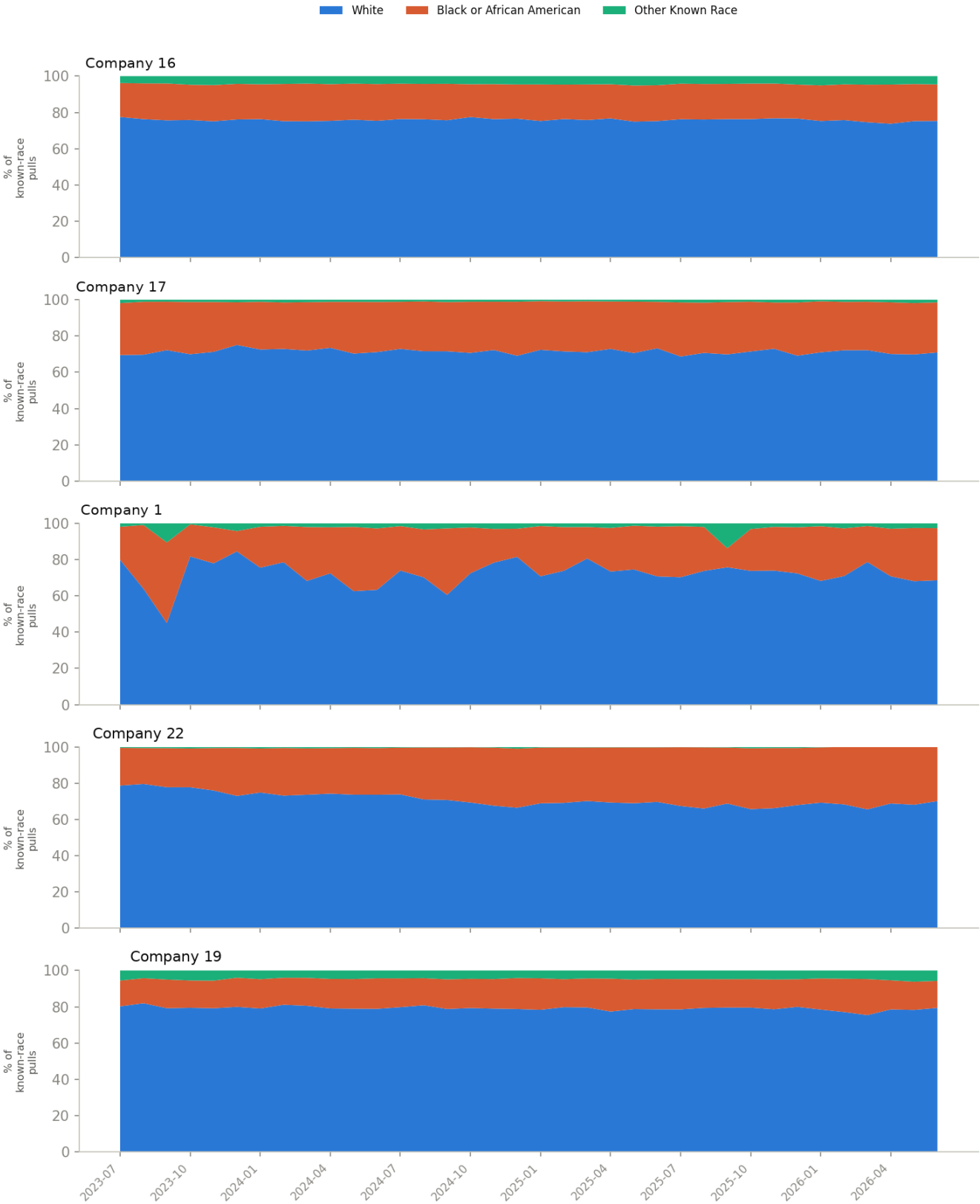


Figure 19. Race composition over time, five largest organizations, as a percent of known-race pulls each month.

6.6 System-Wide Composition Over Time

Sections 6.3-6.5 test individual organizations against the rest of the network. This section reverses the frame: the network as a whole, every organization combined, tracked over the same 36 months - the macro context against which every organization-level finding above should be read.

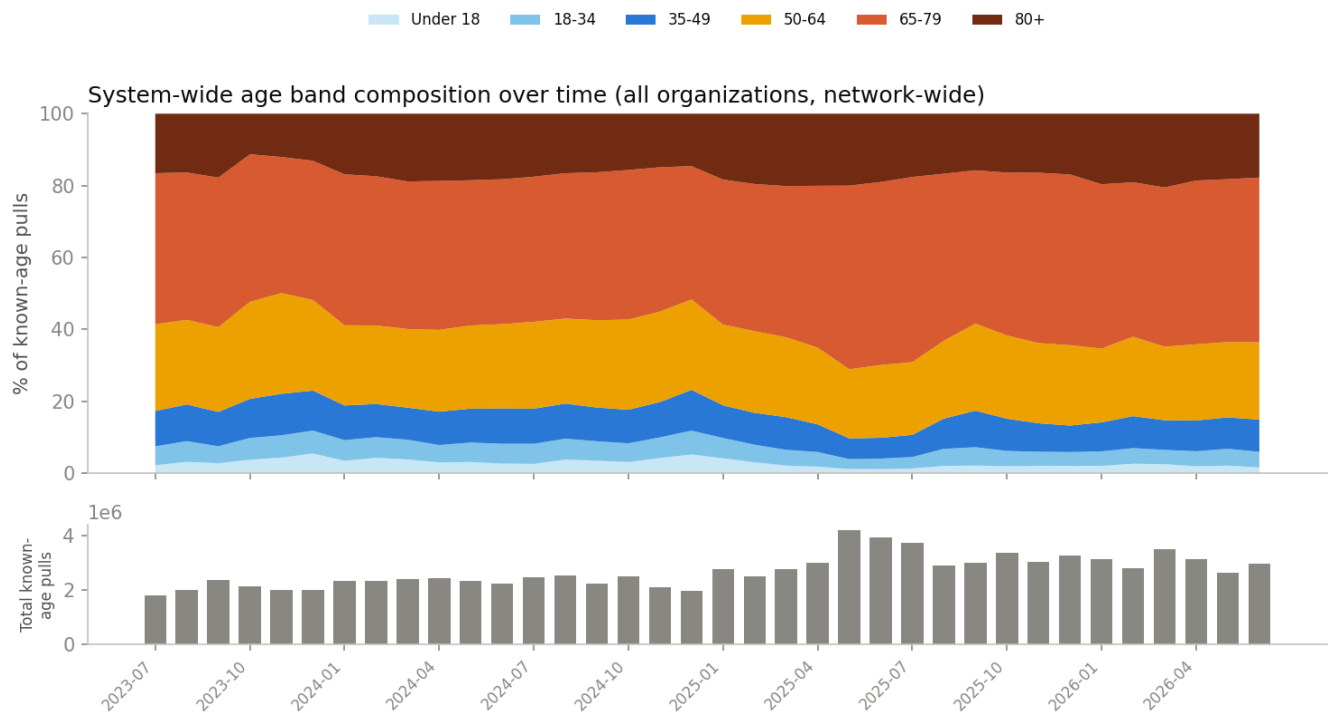


Figure 20. System-wide age band composition over time, all organizations combined, with total known-age pull volume shown below for scale.

System-wide age composition is not static: the 65-79 share, which holds at approximately 40-42% for most of the report window, rises sharply to a peak of 51.5% in July 2025 and stays elevated (45-51%) through most of the remainder of the window, with a corresponding compression in the Under 18, 18-34, and 35-49 shares during the same months. This timing lines up exactly with Company 1's volume surge (Section 3.2.1), and Company 1's own age mix is already known to skew heavily toward 65-79 (Section 6.3). This was tested directly by recomputing the same system-wide series with Company 1 excluded: with Company 1 removed, the 65-79 share stays within a narrower 39.5-45.2% band through 2025, and the July 2025 peak falls from 51.5% to 44.3% - confirming the system-wide shift is substantially a compositional effect of Company 1's surge changing the network's total volume mix, not an independent, network-wide demographic change affecting other organizations.

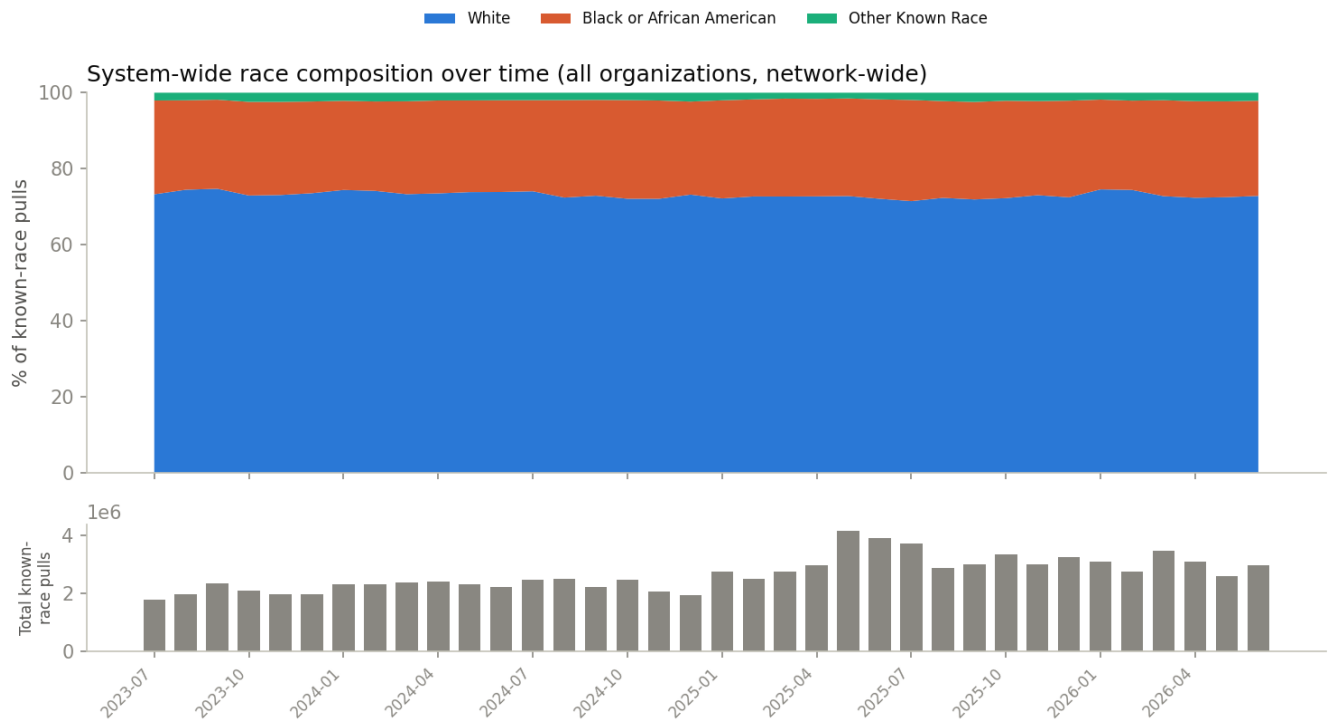


Figure 21. System-wide race composition over time, all organizations combined, with total known-race pull volume shown below for scale.

System-wide race composition, by contrast, is flat across the entire report window - White share stays within 72-75%, Black or African American within 23-27%, and Other Known Race within 1.5-2.4%, with no visible shift coinciding with Company 1's surge, Company 22's decline, or any other volume event in this report. Race composition, at the network level and at every individual organization level tested, shows no evidence of being affected by the volume anomalies this report investigates.

7. Cross-Cutting Hypothesis Worth Testing Directly

Four of the eight additional flagged organizations from the original nine-organization review - Company 2, Company 29 (from the secondary watch list in Section 3.7), Company 9, and Company 22 - are State 1-region entities with unusual patterns clustered in the same general period. That is unlikely to be coincidence at this frequency, and points toward a single shared cause (a common regional HIE, EHR, or billing vendor change) rather than four unrelated organizational decisions.

This revision adds a second, independent data point supporting the same hypothesis: Company 3 and Company 14, discussed in Sections 3.2.3 and 3.5 respectively, show nearly identical race compositions among their known-race pulls (Section 6.2) despite being flagged for entirely different volume patterns (active surge vs. erratic volatility). Whether this reflects a shared referral source, a shared underlying patient population, or is genuinely coincidental has not been tested directly and should be treated as an open question rather than a conclusion.

8. Potential Causes, by Pattern

8.1 Sustained Surges (Company 1, Company 2, Company 3)

- Risk-adjustment or HCC coding chart-retrieval campaigns, consistent with a known industry-wide pattern among Medicare Advantage payors.
- New or changed system integration producing a sudden, sustained volume increase unrelated to a change in clinical need.
- Contract or data-sharing agreement expansion driving a real, durable increase in legitimate volume.
- Misconfiguration or duplicate pulls, a stronger candidate for Company 2 given the mismatch between organization type and pull scale.

8.2 Episodic Spikes (Company 6, Company 7)

- Time-boxed initiatives such as annual wellness-visit documentation drives or quality-measure season chart pulls.
- Scheduled batch or manual export jobs, supported by the all-or-nothing monthly pattern observed.

8.3 Connection Lifecycle Events

- Onboarding backfill for the historical brief connections (Company 9, Company 10).
- Deliberate deactivation or broken integration for the newly identified recently-discontinued connections (Company 11, Company 12, Company 13) - these appear to have functioned normally before stopping, unlike the onboarding-then-silence pattern above.

8.4 Erratic Organizations (Company 14, Company 15)

- An unreliable or manually-triggered scheduled feed is a stronger match for this pattern than either organic demand or a single discrete event.

8.5 Large-Scale Organizations (Company 16, Company 17)

- No corrective action indicated; high statistical scores are a function of scale, not behavior change.

8.6 Company 22 (Decline)

- A lost or narrowed data-sharing agreement, a technical integration problem silently reducing successful pull volume, or activity migrating to a different, differently-named organization ID, per the original finding in Section 3.6.3.

9. Does the Underlying Causal Factor for Company 1 Apply to the Broader Medicare Advantage Industry?

A possible factor driving Company 1's shift in operations is external pressure from policy changes by CMS following the 2025 presidential transition. To this end, a pair of ideas are presented:

- 1) If other Medicare Advantage and CMS plans show a similar utilization shift when measured, an underlying market or policy factor is the more likely explanation; this would point to adjusted data-ingestion practices driven by conditions affecting the industry as a whole rather than a decision specific to Company 1. Other market factors could still produce an industry-wide shift, but a policy change would be the most likely driver given the timing and the companies involved.
- 2) If no similar pattern appears elsewhere, the shift is, in our assessment, attributable to an internal policy change at Company 1 specifically. Whether that internal change was itself a response to external pressure is a separate question and doesn't change this conclusion: at that point, the shift originates within Company 1.

9.1 Method

Twenty-one organizations were identified as Medicare Advantage plans or plan-affiliated entities, cross-referenced against CMS's published list of Medicare Advantage Organizations participating in the CY2025 Value-Based Insurance Design Model and the largest national Medicare Advantage carriers by enrollment (CMS and KFF, 2025-2026):

- Company 1 and Company 34
- Eight Company 20 regional plans
- Company 35 (including its Company 24 and Company 36 operations) and Company 37
- Company 38 (two connections)
- Company 39
- Company 40
- Company 41
- Company 42
- Company 43
- Company 44
- Company 45

Every one of these twenty-one organizations is measured below - none were excluded, and none were selected because of how they scored on the metric being tested.

9.2 Results

Of the twenty-one organizations, seven have negligible or zero volume throughout the report window and provide no usable signal either way. Of the remaining fourteen (including Company 1), thirteen show a Q3 FY25 ratio between 0.74x and 1.33x against their own pre-period baseline; essentially statistical noise clustered around as opposed to a phase-shift in any direction. Company 1 alone shows a 19.0x increase in the same window.

Organization	Pre-Period Avg/Mo	Q3 FY25 Avg/Mo	Ratio
Company 1	100,082	1,902,983	19.0x
Company 35 (incl. Company 24, Company 36)	43,650	32,221	0.74x
Company 43	6,386	8,502	1.33x
Company 37	3,163	3,708	1.17x
Company 20 Georgia	1,380	1,096	0.79x
Company 20 Southern California	1,232	1,168	0.95x
Company 20 Northern California	901	845	0.94x
Company 20 Mid-Atlantic States	651	635	0.97x
Company 41	272	231	0.85x
Company 20 Hawaii	239	227	0.95x
Company 20 Northwest	339	304	0.90x
Company 20 Colorado	306	333	1.09x
Company 20 Washington	183	197	1.07x
Company 45	64	62	0.96x

Table 10. The 13 Medicare Advantage organizations with measurable volume changes, Q3 FY25 phase-shift measurement.

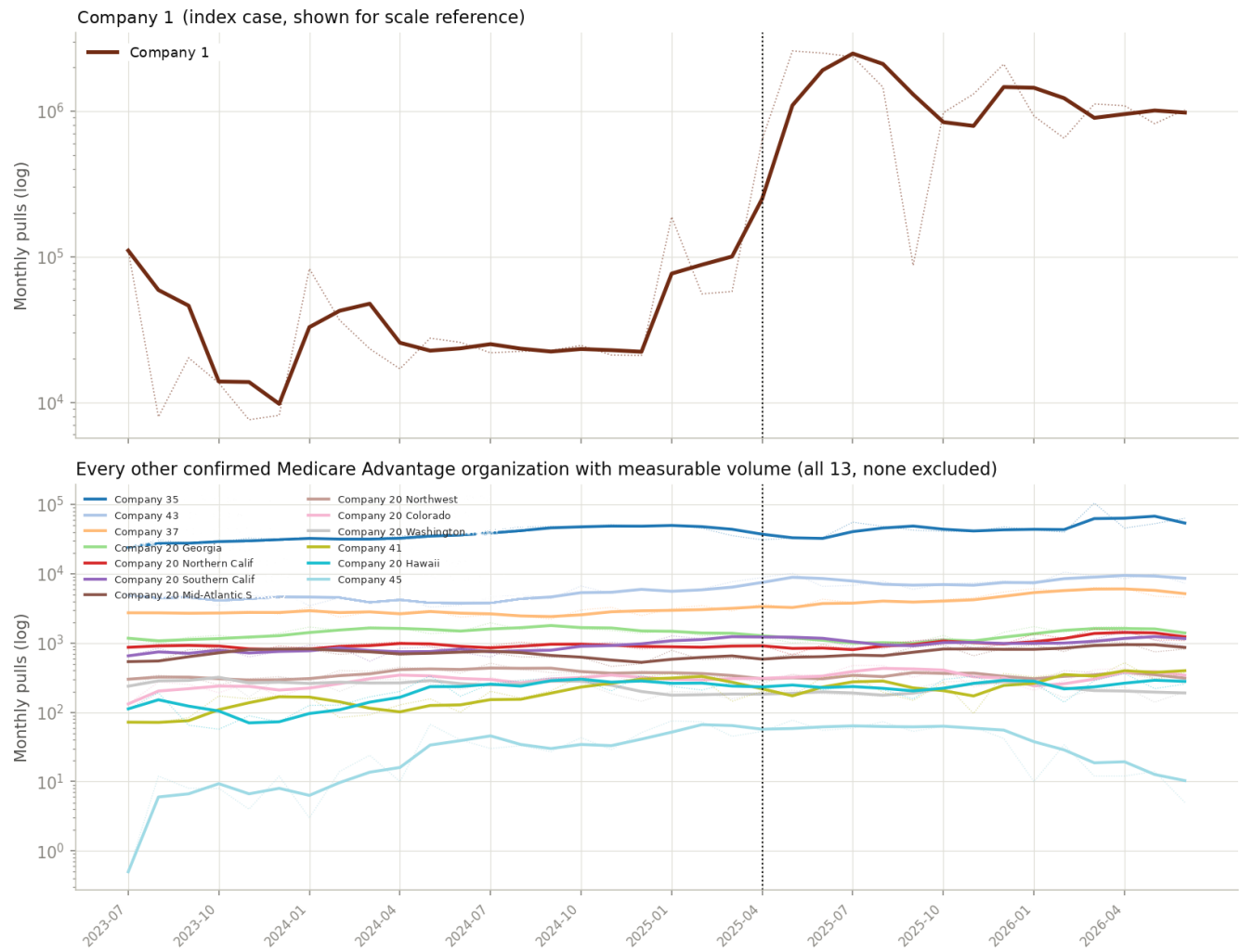


Figure 22. Company 1 (index case) vs. every other Medicare Advantage organization with measurable volume, log scale, raw monthly (dotted) and trailing 3-month average (solid).

9.3 Hypothesis Determination

Two hypotheses were proposed to interpret this section's findings: (1) if other Medicare Advantage plans show a similarly-timed spike, the most likely explanation is an industry-wide adjustment in data-ingestion practices tied to policy demands under the current federal administration; (2) if no such pattern appears elsewhere, the change is most likely internal and specific to Company 1, regardless of what may have motivated it internally.

Restricted to confirmed Medicare Advantage organizations - the comparison class Hypothesis 1 specifies - the evidence supports Hypothesis 2 without qualification. Twenty other Medicare Advantage organizations were measured, including eight independent Company 20 regional plans and Company 35's Company 24 and Company 36 operations, two of the largest Medicare Advantage-affiliated entities in the country. None of these twenty organization exhibits anything resembling Company 1's pattern; ratios sit in a tight 0.74x-1.33x band indistinguishable from ordinary month-to-month variation. If an external, policy-driven shift in data-ingestion practice were occurring across Medicare Advantage plans broadly, a cause that general would be expected to leave a detectable mark somewhere in a comparison group this large. It leaves none.

The finding is unambiguous: this is a change specific to Company 1, not a Medicare Advantage industry-wide pattern.

10. Recommended Next Steps

- Confirm pull legitimacy for Company 1 and Company 3: where unique-patient identifiers are available in source logs, compare pulls-per-unique-patient before and after each surge start date.
- Check Company 2's and Company 7's Care Everywhere connection configuration for signs of a misconfigured recurring job.
- Confirm whether Company 9 and Company 10 were consolidated into Company 22 or another active State 1-region organization ID, rather than treating their silence as unexplained churn.
- Follow up on Company 6's near-zero volume since March 2026 and Company 22's 72% decline since mid-2025; confirm in both cases whether this reflects an intentionally deactivated connection or a broken integration that should be restored.
- Confirm with Company 11, Company 12, and Company 13 whether their connections were intentionally deactivated or represent a broken integration warranting restoration.
- Investigate Company 14's and Company 15's irregular volume as a potentially unreliable scheduled data feed.
- Treat Company 16 as a scale-driven outlier in internal discussion and reporting, distinct from the Tier 1 surge organizations.

11. Limitations

- Demographic and encounter-type coverage is partial and uneven across organizations; see the coverage caveat in Section 6.
- REQ_STATUS_C code meanings beyond code 3 (completed) are not confirmed against an authoritative lookup table; the correct reference table was not located during this analysis.
- Organization names are not deduplicated across naming variants; related entities (for example, the several Company 3-affiliated connections) may be under- or over-counted relative to a fully consolidated corporate view.
- Data is aggregated to organization, date, encounter type, document type, status, and demographic dimensions for analysis; no patient-identifying information is included in this report or its underlying exports.